Maternal mortality in fragile and conflict-affected situations: technical brief

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Reducing maternal mortality is one of nine targets for Sustainable Development Goal 3 (SDG3): "Ensure healthy lives and promote well-being for all at all ages". While many countries and territories have made progress in reducing maternal mortality, women in some parts of the world are at heightened risk of being left behind. Of the estimated 260 000 maternal deaths that happened globally in 2023, as reported in the ***Trends in maternal mortality estimates 2000 to 2023,*** around 6 in 10 (61%; 160 000 deaths) took place in countries and territories experiencing conflict or institutional and social fragility, as defined by the World Bank (*1–3)* (see **Fig. 1**).

Fragile and conflict-affected settings are dynamic in nature – some countries fluctuate between stability and fragility, including those that are particularly susceptible to natural disasters and the effects of climate change. The term "conflict-affected" here refers to countries identified by the World Bank based on the intensity of the violence and the number of conflict-related deaths relative to the population. Meanwhile, "institutional and social fragility" includes countries identified based on the quality of governance, policies and institutions, as well as specific manifestations of fragility, such as the presence of a United Nations peacekeeping operation or population flight across borders seeking international protection.

The 17 countries and territories classified as experiencing conflict by the World Bank in 2023 were home to 10% of the global population of women of reproductive age, 21% of all live births, and 55% of all maternal deaths. The 20 countries and territories classified as experiencing institutional and social fragility were home to just 2% of all women of reproductive age, 4% of all live births and 7% of all maternal deaths (see **Fig. 2**).

According to the ***World Bank Group strategy for fragility, conflict, and violence 2020–2025 (Vol. 1)***, the fragility landscape has worsened significantly in recent years, impacting low- and middle-income countries experiencing social and economic crises and persistent poor governance, contributing to rising inequality, lack of opportunity, discrimination and exclusion *(4)*. These factors can increase vulnerability to shocks and crises, and create regional and global spillovers. The impacts can be devastating, particularly for women, children, young people and people with disabilities, and can be felt for generations.

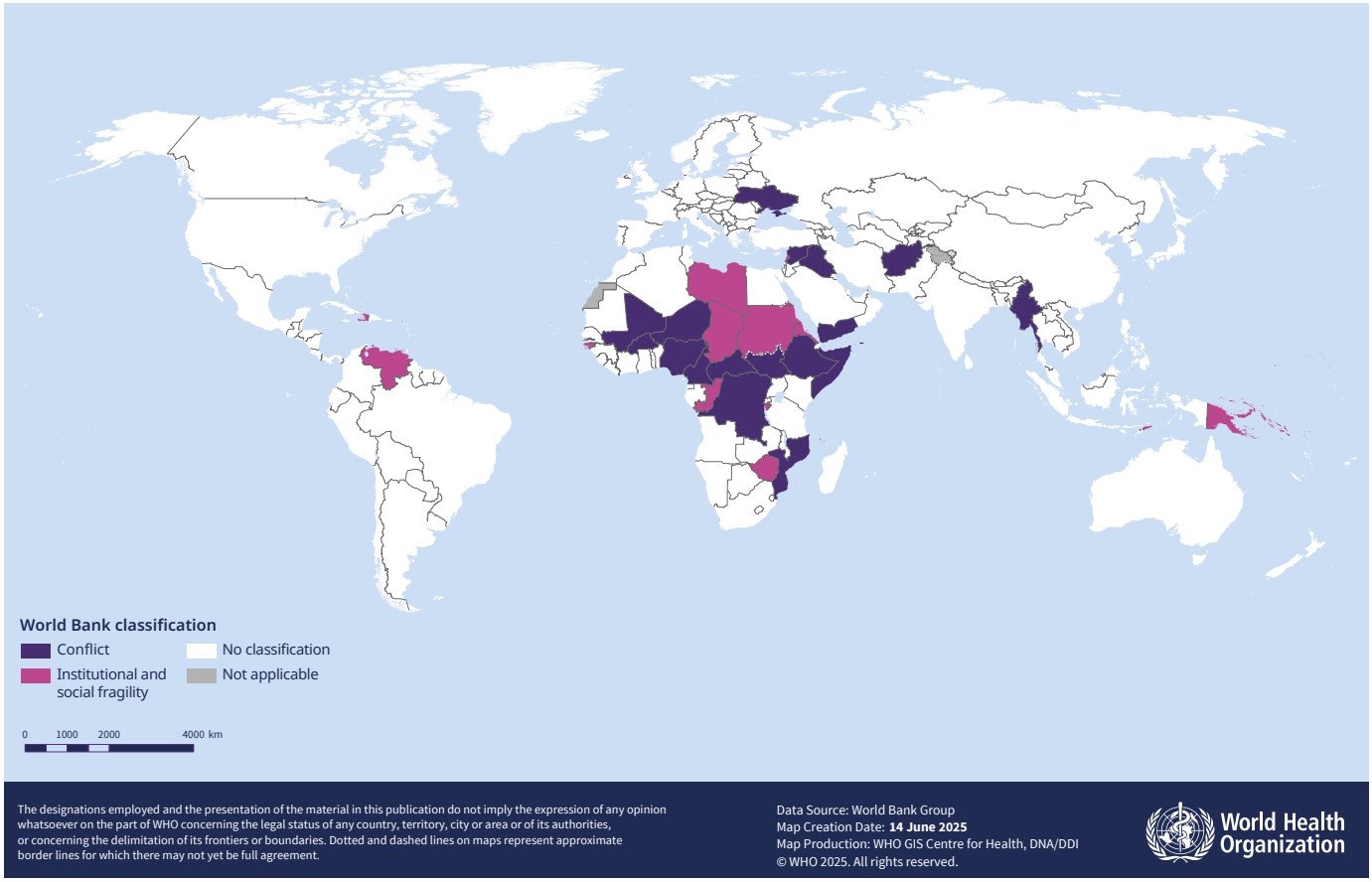

Fig 1. World Bank classification of countries and areas, 2023

All SDG regions apart from Australia and New Zealand were affected by conflict or institutional and social fragility in 2023. Many of the countries and territories experiencing conflict or institutional and social fragility were in the sub-Saharan Africa region (18 countries and territories), followed by Northern Africa and Western Asia (seven countries and territories) and Oceania (excluding Australia and New Zealand) (five countries and territories) (see **Table 1**).

Greater risk of death

Conflict and fragility affect the stability of health systems and their ability to provide quality health services. During emergencies and humanitarian crises, for example, there is often an increased need for health services among populations. At the same time, the routine functioning of health systems is disrupted by unpredictable and unsafe conditions for health workers and health service users; damage to or destruction of physical infrastructure; targeting of health facilities, health workers or ambulances; loss of trained health workers; and disrupted supply chains. In these settings, multiple challenges converge to obstruct access to and decrease the quality of maternal health services. Pregnant women, mothers and babies often lack access to trained health workers, basic and emergency obstetric and newborn care, and essential postnatal services, putting both mothers and newborns at heightened risk of adverse outcomes (read about examples of this in the case studies from Colombia, Ethiopia, Haiti, Myanmar, Papua New Guinea and Ukraine).

In 2023, the estimated number of maternal deaths relative to the number of live births – referred to as the maternal mortality ratio (MMR) – was very high in countries affected by conflict (504 maternal deaths per 100 000 live births, with an 80% uncertainty interval [UI] ranging from 413 to 674), and it was also high in countries experiencing institutional and social fragility (368 maternal deaths per 100 000 live births; UI: 300–473). The estimated MMR was low in countries that were not experiencing conflict or fragility (99 maternal deaths per 100 000 live births; UI: 91–110). For further details about these estimates, refer to Chapter 3 (Methods) in ***Trends in maternal mortality estimates 2000 to 2023*** *(2)*.

Across all SDG regions in 2023, maternal mortality was higher in settings experiencing conflict and institutional and social fragility than in settings not classified in either of these categories (see **Table 1**). Conflict-affected countries in sub-Saharan Africa had a very high MMR (571; UI: 460–782) while countries in sub-Saharan Africa that were not experiencing conflict or institutional and social fragility had a moderate MMR (268; UI: 238–313). Similar differences between countries experiencing conflict and those that were not could be seen in all other affected SDG regions.

Reducing excess mortality (i.e. the difference between the number of deaths occurring due to a crisis and the number of deaths expected during "normal" circumstances for the same population), including excess deaths from maternal mortality, is a cornerstone of the humanitarian imperative to save lives, alleviate suffering and restore dignity. Conflict and fragility can lead to displacement and forced migration with large parts of the population spending long periods of time away from their homes in refugee camps, settlements or other unfamiliar settings where they are highly vulnerable to poor health outcomes, including increased risk of maternal mortality. The intersection of gender, ethnicity, age and migration status can increase the vulnerability of women and girls in these contexts. A 15-year-old girl living in a country or territory affected by conflict in 2023 had a 1 in 51 lifetime risk of eventually dying from a maternal cause, compared with a 1 in 79 risk for a 15-year-old girl living in a country or territory affected by institutional and social fragility, and 1 in 593 for a 15-year-old girl living in a country or territory not classified as experiencing conflict or institutional and social fragility.

Health services in complex settings

For women to realize their right to sexual and reproductive health (SRH), it is essential that they can access SRH services, including emergency obstetric care and skilled health personnel to provide care during childbirth, to help lower their risk of maternal mortality and morbidity *(5)*. Providing maternal health services to women in settings affected by conflict or fragility, including

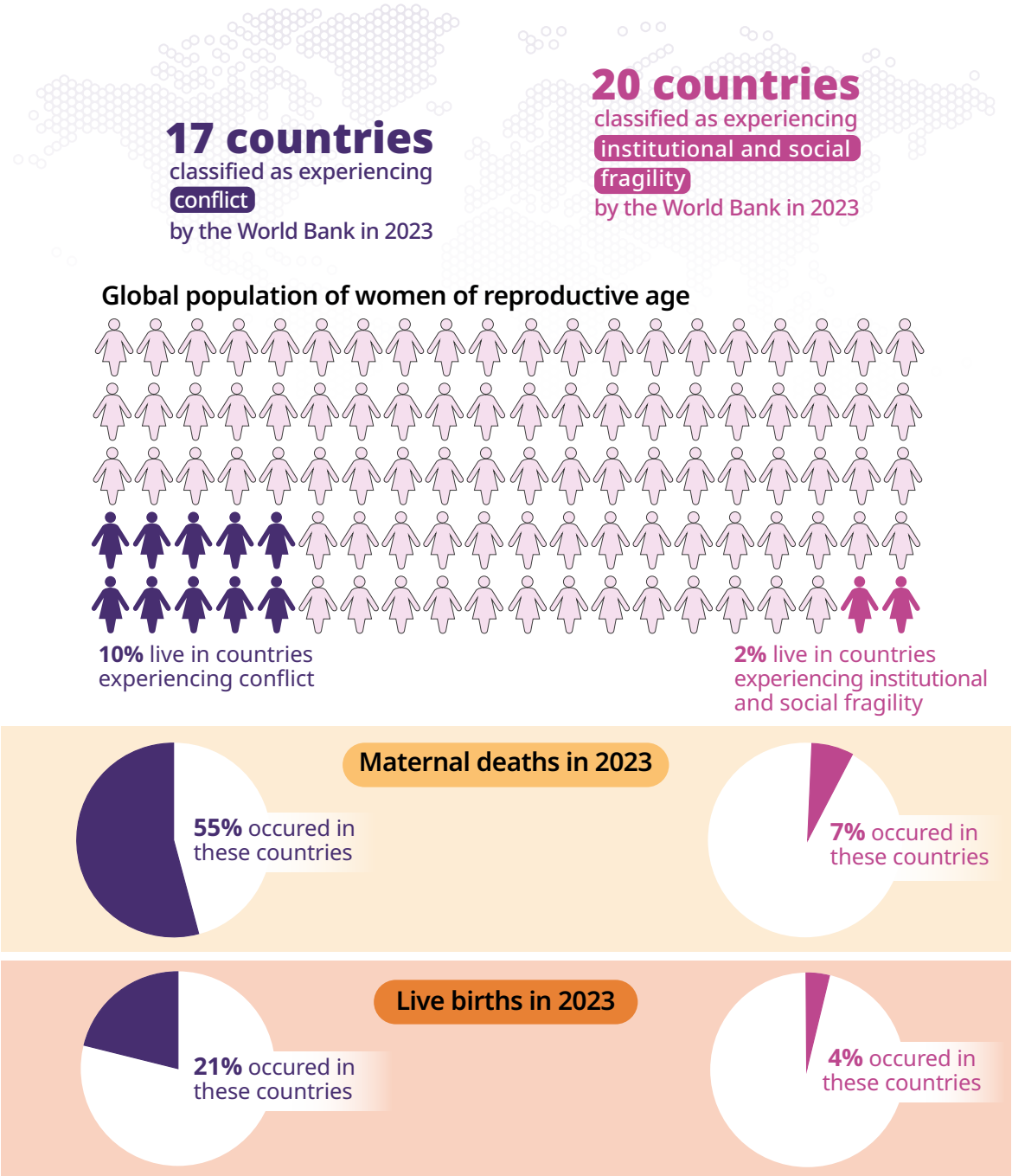

Fig. 2. Proportion of all maternal deaths and live births by World Bank classification of countries, 2023

internally displaced persons (IDPs) and refugees, is a complex task, which, if neglected, has serious consequences, including maternal deaths that could have been prevented. WHO's package of high-priority health services for humanitarian response (H3 package) *(6)* outlines the core SRH services that should be delivered as a minimum during protracted humanitarian emergencies, including a dedicated pregnancy and birth care facility and provision of basic emergency obstetric and newborn care, in close alignment with the WHO UHC Compendium *(7)* and UHC Service Package Delivery and Implementation (SPDI) tool *(8)*. Additionally, the Inter-Agency Working Group on Reproductive Health in Crises (IAWG) created the Minimum Initial Service Package (MISP) for Sexual and Reproductive Health *(9)*, which includes provision of emergency obstetric and neonatal care and contraception in the set of SRH service-delivery activities that are most important in preventing morbidity and mortality during the early phases of acute humanitarian emergencies, while ensuring that the activities are sensitive to the health needs of women, mothers and newborns in these settings, and also supporting the move to comprehensive reproductive health services as soon as feasible.

Table 1. Maternal mortality ratio (MMR) by SDG region and World Bank classification of countries, 2023

SDG region	World Bank classification	No. of countries (% out of region)	MMR point estimate (80% UI)
Australia and New Zealand	Conflict	0	NA
	Institutional & social fragility	0	NA
	No classification	2 (100)	3 (2–4)
Central and Southern Asia	Conflict	1 (7)	521 (339–942)
	Institutional & social fragility	0	NA
	No classification	13 (93)	94 (83–111)
Eastern and South-Eastern Asia	Conflict	1 (6)	185 (124–311)
	Institutional & social fragility	1 (6)	192 (125–228)
	No classification	14 (88)	59 (47–81)
Europe and Northern America	Conflict	1 (2)	15 (9–25)
	Institutional & social fragility	0	NA
	No classification	43 (98)	11 (9–12)
Latin America and the Caribbean	Conflict	0	NA
	Institutional & social fragility	2 (5)	277 (199–404)
	No classification	32 (95)	60 (54–68)
Northern Africa and Western Asia	Conflict	3 (13)	85 (59–132)
	Institutional & social fragility	4 (17)	234 (146–375)
	No classification	17 (70)	28 (24–32)
Oceania (excluding Australia and New Zealand)	Conflict	0	NA
	Institutional & social fragility	5 (38)	185 (121–293)
	No classification	8 (62)	74 (50–123)
Sub-Saharan Africa	Conflict	11 (22)	571 (460–782)
	Institutional & social fragility	7 (15)	517 (404–715)
	No classification	30 (62)	268 (238–313)

The WHO Global Health Cluster Sexual and Reproductive Health Task Team *(10)* was established in November 2022 with a goal to ensure that SRH priorities are systematically addressed in all phases of any humanitarian response and that SRH coordination is consistently included in cluster coordination at both the global and country levels. Key outputs of the SRH Task Team are to strengthen, systematize and standardize SRH coordination at the country level; contribute to the reduction of maternal and newborn mortality; contribute to the prevention of unwanted pregnancies; and develop effective linkages between activities addressing SRH and gender-based violence.

WHO country offices in settings affected by conflict or other forms of fragility work closely with partners to reduce preventable maternal mortality.

- In **Colombia** this has been done through supporting traditional birth attendants and providing culturally adapted maternal health care.
- In **Ethiopia** this has been done through strengthening the health system to improve the availability of life-saving services.
- In **Haiti** this has been done by negotiating to increase access to and quality of maternal health services in hospitals.
- In **Myanmar** this has been done by advocating for sexual, reproductive, maternal, newborn, child and adolescent health (SRMNCAH) services and creating subnational plans for maternal mortality reduction.
- In **Papua New Guinea** this has been done through quality improvement interventions to help women feel safe and respected while delivering babies.
- In **Ukraine** this has been done through supporting improvement of infection prevention and control and microbiological diagnostics in maternity hospitals.

Refer to the case studies in this brief for further details.

Recalling global commitments

Across the world, the number of conflicts is rising, and they are becoming increasingly entrenched. In 2024, the Office of the United Nations High Commissioner for Refugees (UNHCR) estimated that 122.6 million people – mostly women and children – remained forcibly displaced worldwide due to conflict, persecution, violence and human rights violations, while an estimated 32 million women and girls of reproductive age were living in emergency situations *(11)*.

SDG target 3.1 aims to reduce the global MMR to less than 70 maternal deaths per 100 000 live births, with no country having an MMR greater than 140 deaths per 100 000 live births, by 2030. According to the report *Trends in maternal mortality 2000 to 2023 (2)*, the world is off track to achieve this goal. If the same rate of MMR reduction observed in the first half of the SDG era (2016 to 2023) continues for the remainder of the era, 45 countries will still have an MMR above 140 in 2030, and over half of those (23) are countries currently classified as experiencing conflict or institutional and social fragility in 2023 (see **Fig. 3**).

Resilient health systems have the capacity to anticipate, absorb, adapt to and recover from crises, while ensuring continuity and quality of maternal and SRH services *(12)*. Milestone 7, of the 10 milestones proposed to track implementation of the Ending Preventable Maternal Mortality (EPMM) strategies to accelerate progress for maternal health and well-being in high-burden settings, covers "response and resilience" *(13)*. This milestone is reached when "all countries have a preparedness and response plan that includes promoting maternal and newborn survival and health and preventing stillbirths and have a coordinated mechanism in place for its implementation, ensure procurement of emergency supplies and monitoring survival and health outcomes".

While operational challenges restrict research on maternal health in fragile and conflict-affected settings and limit the capacity to monitor progress (see **Box 1**), it is essential that strategies to protect

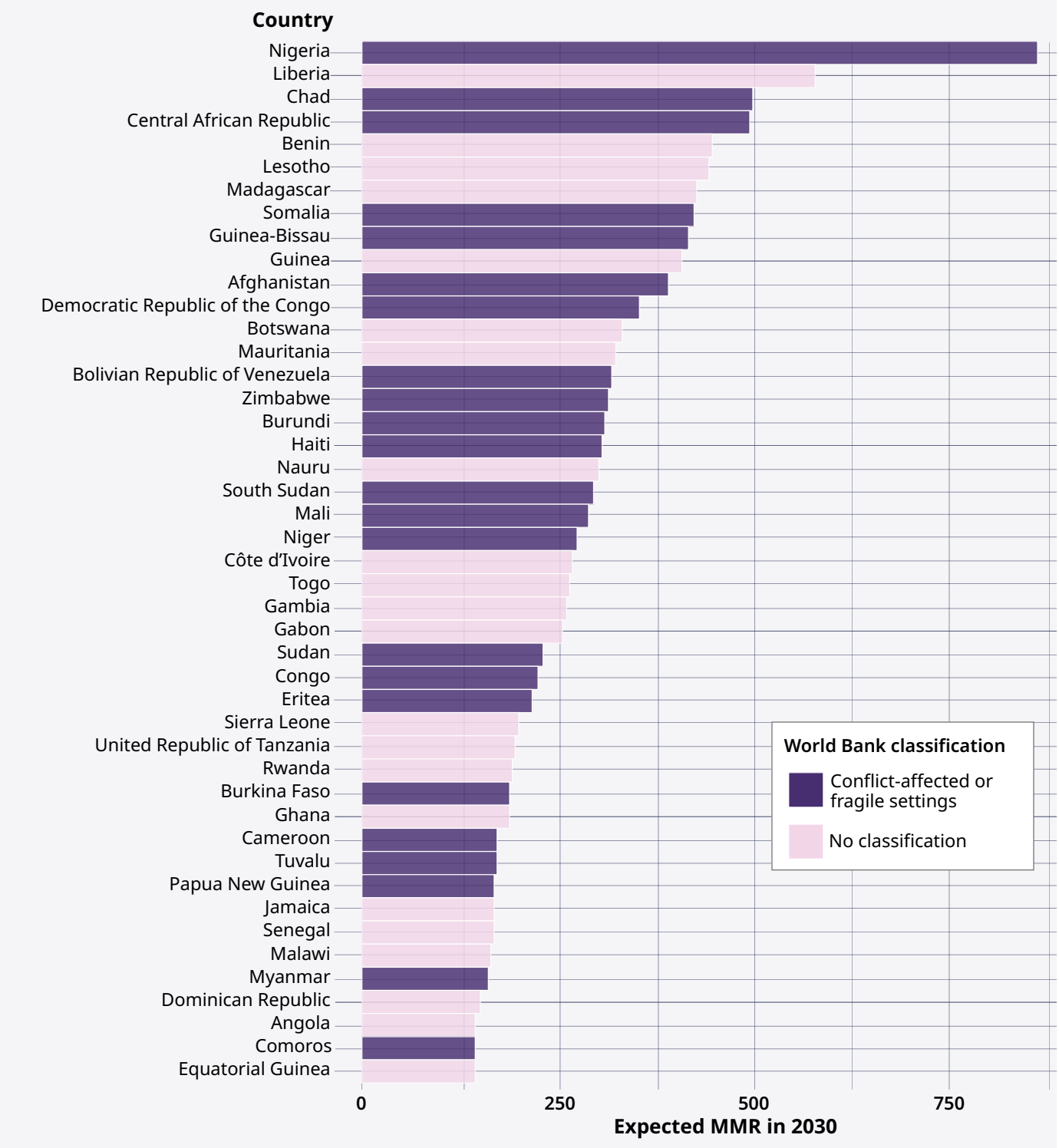

Fig. 3. Countries expected to have maternal mortality ratio (MMR) above 140 in 2030 assuming continuation of the average annual rate of reduction between 2016 and 2023 (countries listed from highest to lowest MMR)

maternal health incorporate ways of building up the resilience of health systems against the impacts of conflict and other potential shocks.

A resolution was passed at the Seventy-seventh World Health Assembly in May 2024 to accelerate progress towards reducing maternal, newborn and child mortality in order to achieve SDG targets 3.1 and 3.2. This resolution recognized that over half of maternal, newborn and child deaths globally occur in fragile and humanitarian settings, and it highlighted the urgent need to increase investments to expand coverage and improve the quality of primary health care services in

fragile contexts *(14)*. In 2025, 307.6 million people are estimated to require urgent humanitarian assistance and protection worldwide as multiple crises escalate with devastating consequences for the people affected by them, particularly in the regions of East and Southern Africa, the Middle East and North Africa, and West and Central Africa, as described in the ***Global humanitarian overview 2025*** *(15)*.

If the world is to achieve SDG 3.1, it is vital to improve both access to and quality of maternal health services in fragile and conflict-affected settings. Improving health facility capabilities, optimizing the health workforce, and sustainably financing health services can increase the strength and resilience of health systems to external shocks and disruptions, such as climate change, conflict, post-conflict and disaster situations, all of which threaten the health and well-being of individuals and progress towards global health goals.

Box 1. Data challenges

Reliable vital statistics on the numbers and distributions of births, deaths and causes of death are essential to identify and address inequalities and track progress towards global health goals. It is important to note, however, that while the estimates presented in this technical brief can serve as a valuable guide for policy-making and programme planning, many women who die from maternal causes are still not being counted. In many countries experiencing conflict or institutional and social fragility, mortality data are sparse or – when they are available – of low quality, making accurate measurement difficult. The maternal mortality estimates do not account for variations in mortality resulting from local crises within countries and should be interpreted with this in mind.

Key messages

- ☑ Around 6 in 10 maternal deaths in 2023 took place in countries and territories experiencing conflict or institutional and social fragility.
- ☑ The estimated number of maternal deaths relative to live births (the MMR) was very high in settings experiencing conflict in 2023 (504 maternal deaths per 100 000 live births) and high for those living in settings experiencing institutional and social fragility in 2023 (368).
- ☑ With an increasingly uncertain fragility landscape, bolstering health system resilience against conflict, climate change and other potential shocks is essential to protect maternal health and save lives.
- ☑ Ramping up investment in primary health care can improve access to and quality of maternal health services in fragile and conflict-affected settings – sustainable financing will make health systems stronger and more resilient.
- ☑ Improving data collection on maternal and newborn mortality at the subnational level, particularly in settings where humanitarian access is restricted, can help identify and address inequities and track progress towards ending preventable maternal deaths.

Fatioma, 30, a pregnant refugee who fled conflict in Sudan, sits with her daughter in front of their shelter in a camp in Adre, Chad.© WHO/ Nicolò Filippo Rosso

Country case studies

Case study 1

Culturally adapted maternal health care in Colombia 10

Case study 2

Ensuring maternal health during conflict in Ethiopia 11

Case study 3

Saving one maternal life at a time in Haiti 13

Case study 4

Multiple crises converge to impact maternal mortality in Myanmar 14

Case study 5

Tackling maternal mortality in Papua New Guinea 15

Case study 6

Maternal health care resilience during conflict in Ukraine 17

Middle Atrato, by the Bojaya River, Chocó, Colombia
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Case study 1

Culturally adapted maternal health care in Colombia

Colombia had a low MMR in 2023, with 50 maternal deaths per 100 000 live births. However, in parts of the country the MMR is several times higher than the national figure, including in departments affected by natural disasters such as drought and floods, and where guerrillas and paramilitary groups impact daily life, such as Chocó and Guajira.

The Pan American Health Organization (PAHO) is working to reduce maternal mortality in these areas in close collaboration with populations experiencing insecurity as part of the ***Plan de aceleración para la reducción de la mortalidad maternal*** (National acceleration plan for maternal mortality reduction) and emergency project. In remote and conflict-affected areas where access to birth facilities is limited, community members act as traditional birth attendants and have formed associations to share their ancestral knowledge with younger generations of birth attendants. To facilitate the dissemination of knowledge among traditional birth attendants, PAHO organized training-of-trainers sessions for members of traditional birth attendant associations.

PAHO provided kits to traditional birth attendants, containing tools to monitor pregnant women, identify those at risk of maternal morbidity and mortality, and make timely referrals. The kit includes a stethoscope, blood pressure monitor, oximeter, measuring tape, scales, anti-shock garments for postpartum haemorrhage, and guidance on referring women to health facilities and hospitals. Additionally, traditional birth attendants and community leaders have been trained to participate in community-based surveillance of public health events, including maternal and neonatal deaths.

In some communities, rejection of Western medicine has led to delays in seeking medical care. To help address this, PAHO is supporting cultural adaptation of hospital maternity wards for Indigenous people, respecting Indigenous ancestral knowledge and birth practices.

These ethnic and gender-sensitive interventions can improve access to sexual, reproductive, maternal and neonatal health services for vulnerable women by strengthening community and social networks.

Antenatal care visit performed by a trained health worker, Ethiopia.

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Case study 2

Ensuring maternal health during conflict in Ethiopia

Maternal health has been improving in Ethiopia recently, with the MMR decreasing from 267 to 195 maternal deaths per 100 000 live births between 2020 and 2023. However, the humanitarian crisis in various regions of the country has severely disrupted health-care services.

Pregnant women in the areas affected by conflict, drought and displacement, particularly Tigray, Amhara, Afar and Oromia regions, face significant challenges in accessing essential maternal health services, including antenatal care, skilled health personnel to provide care during childbirth, and emergency obstetric care.

Lack of ambulances and restrictions on vehicle movements at night have exacerbated the problems for pregnant women trying to reach health facilities and contributed to an increase in the number of home births. Furthermore, many health workers have fled conflict-affected areas. Restoring maternal health services remains an ongoing challenge such that there is an increased risk of complications and maternal death.

In response, WHO, the United Nations Children's Fund (UNICEF) and the United Nations Population Fund (UNFPA), with funding support from the Bill & Melinda Gates Foundation, launched the Service Delivery Innovations in Conflict-Affected Areas (SDI) project in selected conflict-affected zones of Amhara, Oromia and Afar. The project aims to enhance maternal, neonatal and child health services and strengthen health systems in three zones and 12 districts of Afar, Amhara and Oromia. Collaborating with Ethiopia's Ministry of Health and Regional Health Bureaus, the agencies established tailored implementation and coordination mechanisms to improve the availability of life-saving reproductive, maternal, newborn, child and adolescent health and nutrition (RMNCAH-N) services aligned with local health service delivery systems.

Activities have included: distribution of three ambulances; deployment of six mobile health service teams for underserved and remote communities; renovation of 24 maternity waiting homes, six neonatal intensive care units and four maternity wards; training and deployment of 24 midwives; increasing the capacity of health workers; providing

essential medicines, equipment and supplies; and improving monitoring and evaluation, and evidence-generation.

As part of the project, WHO, the Ministry of Health, the Ethiopian Public Health Institute and other stakeholders adapted guidelines and tools to support SDI models for RMNCAH-N services in conflict-affected areas. These resources aim to bridge skills gaps among service providers and improve the quality of care for mothers, children and adolescents.

The Ethiopian Public Health Institute is coordinating the SRH response at the district level, including identifying the causes of maternal mortality through data collected by the national surveillance system. These data can be used by the Ministry of Health and district offices to direct activities, such as applying MISP in affected populations.

Since the SDI project began in 2022, the number of deliveries attended by skilled health workers increased from 12 790 to 17 620 in 2024. The number of women and girls receiving at least four antenatal care visits rose from 15 636 in 2022 to 23 228 in 2024, with the dropout rate from first to fourth visit decreasing from 32% in 2022 to 17% in 2024. Finally, the number of women receiving postnatal care visits within seven days of delivery increased from 17 611 in 2022 to 21 730 in 2024. These improvements in maternal health service quality and uptake reflect the positive contributions of the project and all people involved.

A nine months pregnant woman attending a routine check-up, Ethiopia.
© UNICEF Ethiopia / Ayene

A midwife provides breastfeeding guidance to a new mother at the Beaumont Health Center in the Grand'Anse department of Haiti.
© PAHO/WHO – David Lorens Mentor

Case study 3

Saving one maternal life at a time in Haiti

In recent years, Haiti has experienced economic, political and social turmoil, and natural disasters, as well as a security crisis which has directly impacted the health system and displaced more than 1 million people in and around the metropolitan area of Port-au-Prince. This has severely impacted access to health services and especially maternal health in a country with the highest MMR in the SDG region of Latin America and the Caribbean (328 maternal deaths per 100 000 live births in 2023).

As a response to the humanitarian emergency created by mass displacement, the Pan American Health Organization (PAHO), in support of Haiti's Ministry of Public Health and Population, endeavoured to re-establish access to health care within the many internally displaced persons (IDP) sites in and around the capital city. As part of the team deployed on the ground, obstetric nurses and community health workers carried out quick assessments to identify women with high-risk pregnancies and refer them to the nearest hospital to improve maternal and neonatal outcomes. In coordination with the United Nations Population Fund (UNFPA), PAHO partnered with three hospitals in the metropolitan area of Port-au-Prince to provide free caesarean sections for women living in IDP sites and at half the cost for other women. PAHO assisted these hospitals through the provision of essential medical equipment and products, including anti-shock garments for postpartum haemorrhage, and the installation of a reliable power supply system, guaranteeing constant availability of electricity.

In the largest of the three hospitals, the Hôpital Universitaire la Paix, 1554 women benefited from this programme in 2024. PAHO and UNFPA have also extended their support beyond maternity services, to respond to other urgent SRH needs; kits for the management of abortion complications and kits for the management of sexual violence have been distributed.

A health worker examines a pregnant woman as part of her antenatal care.
© WHO / Christine McNab

Case study 4

Multiple crises converge to impact maternal mortality in Myanmar

Political turmoil in Myanmar in 2021 led to mass protests and a civil disobedience movement during which many health workers went on strike, resulting in significant disruption to essential health services at a time when they were impacted by the COVID-19 pandemic. Although the situation has somewhat stabilized since then, the country is experiencing an ongoing emergency with escalating conflicts in several states and regions, and over 3 million internally displaced persons. Furthermore, every year Myanmar is vulnerable to various climate hazards, including cyclones, droughts, floods and extreme temperatures.

Gains in maternal mortality achieved on the road towards SDG target 3.1 (reducing maternal mortality) were quickly reversed because of the COVID-19 pandemic, the political crisis and conflict. In 2023, Myanmar had a moderate MMR with 185 maternal deaths per 100 000 live births (with an 80% uncertainty interval ranging from 124 to 311 deaths). With health facilities operating at limited capacity, reduced access to services, supply chain issues and shortages of essential medicines, maternal health services remain unstable.

The WHO Country Office for Myanmar has been working with United Nations agencies and partners to advocate for SRMNCAH services throughout the country and is working with the health authorities to create focused subnational plans to reduce preventable maternal mortality. A national workshop was held to identify key interventions for the main causes of maternal deaths and to determine priority activities to be implemented over the next five years to reduce preventable maternal deaths in all 17 states and regions. Furthermore, during this process, the WHO Country Office has adapted and used the WHO Regional Office for South-East Asia's guides on ***Managing programmes on reproductive, maternal, newborn, child and adolescent health*** (a module-based training course) *(16)* ***and the Manual for a short programme review on the same topic in the South-East Asia Region*** *(17)*.

Conflict, political upheaval, climate change and COVID-19 together have negatively affected Myanmar's chances of achieving SDG target 3.1. The March 2025 earthquake, which directly affected 9.1 million people in Myanmar, further strained the health system, particularly in already underserved areas, disrupting essential maternal health services. It is hoped that subnational plans to reduce maternal mortality will help with course correction over the next five years.

A mother and her newborn infant after delivery thru cesarean section, St Mary Hospital Vunapope, Kopopo, Papua New Ginea.
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Case study 5

Tackling maternal mortality in Papua New Guinea

Papua New Guinea is an island nation in Melanesia, in the WHO Region of the Western Pacific. In addition to ongoing tribal conflict, Papua New Guinea is vulnerable to the effects of climate change, including floods, earthquakes, landslides and volcanic activity. As a result of the geography and the security situation, accessing health facilities can be challenging, and where services are available, they may be of low quality with facilities in disrepair, lacking water, power supplies and skilled birth attendants. In 2023, 42% of births in Papua New Guinea took place in a health facility and there were 189 maternal deaths per 100 000 live births.

To reduce maternal mortality, since August 2024 the WHO Country Office for Papua New Guinea has been supporting Port Moresby General Hospital, Modilon Provincial Hospital in Madang, and Buka General Hospital in the Autonomous Region of Bougainville to improve the quality of basic intrapartum and early postpartum care. This has involved adapting a WHO Western Pacific Regional Office initiative for continuous quality improvement to the national context and collaborating with the National Department of Health, professional societies, health workers and clients to jointly review existing intrapartum and postpartum care practices, identify service gaps, develop an action plan, provide clinical coaching and implement routine measures for monitoring and improvement.

The overall aim is for all health facilities in Papua New Guinea to institutionalize continuous quality improvement mechanisms so that women feel safe and respected while delivering their babies. Short-term targets include encouraging women to have a companion of choice with them during labour and childbirth, prolonged skin-to-skin contact for at least 90 minutes immediately after birth, and fostering effective communication between health workers and pregnant/postpartum women. Management teams in the three hospitals are helping to create a supportive environment for essential practices, including consistent hand hygiene.

When the initiative began, only half of babies born in Port Moresby General Hospital had skin-to-skin contact with their mothers immediately after birth. Six months later, all mothers and newborns were experiencing this crucial practice. Moreover,

the promotion of exclusive breastfeeding and protection from infection and hypothermia are increasing newborn survival rates. At the start of the initiative, the absence of effective stock management led to the expiration of some life-saving medicines. However, after six months, an effective management system was implemented, ensuring no further stock-outs of essential medicines for childbirth since then.

Better quality intrapartum and early postpartum care in health facilities can encourage women to seek health services, increase service accountability, and reduce complications and referrals. By sharing positive pregnancy experiences, women, their families and community leaders can become aware of the services available in health facilities. Quality of care improvements not only contribute to survival but also enhanced quality of life for both mothers and babies.

Postnatal care of mother after delivery thru cesarean section by a nursing student, St Mary Hospital Vunapope, Kopopo, Papua New Ginea.
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Neonatal service in a maternity ward, Ukraine
© EU / Oleksandr Ratushnyak

Case study 6

Maternal health care resilience during conflict in Ukraine

Since the Russian Federation's invasion of Ukraine in February 2022, Ukraine has been experiencing unprecedented challenges, including provision of health care services to the population. Despite over 2400 attacks on the health system so far, including over 2000 attacks resulting in direct damage to or destruction of hospitals and clinics *(18)*, Ukraine had a very low MMR in 2023, with 15 maternal deaths per 100 000 live births.

The resilience of Ukraine's health system has ensured ongoing availability of maternal health services in areas affected by the war. With financing from the State budget and other external sources, the National Health Service of Ukraine has been able to continue contracting health facilities to provide the same package of maternal health services at the same rate as before the invasion. Furthermore, patient pathways have been modified so that maternity services can be provided in health facilities located in areas not affected by the war.

Across Ukraine, all health facilities have been renovated to provide shelter and ground-floor care for pregnant women during air attacks, with the support of local authorities and donors. In addition to providing continuity of maternal care services, efforts to eliminate mother-to-child transmission of HIV, syphilis and hepatitis in Ukraine continue and new initiatives to improve neonatal outcomes, including universal neonatal screening for 21 congenital diseases, have been launched. The WHO Country Office for Ukraine has been supporting improvement of the infection prevention and control programme as well as microbiological diagnostics in maternal hospitals through capacity-building, training programmes and technical support visits.

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